

Sponsoring Organisation:	Implementation Date:	01 December 2015
DH Informatics Directorate	Subject:	NHS Number Input and Display for Clinical Systems within the NHS in England
DATA SET CHANGE NOTICE		
This DSCN informs users of the approval of a new information standard by the Information Standards Board for Health and Social Care (ISB). It was presented at ISB's meeting on 24 September 2009 and subsequently approved having met the required conditions.		
Summary: This user interface standard specifies the rules for the input and display of NHS Numbers in clinical systems. The current NHS Number Information Standard for General Practice (England) and current The NHS Number Information Standard for Secondary Care (England) should also be referred to when reviewing this Standard. Organisations developing or upgrading applications for which the standard is in scope MAY implement the NHS Number input and display standard immediately and SHOULD do so at their next major system release. All clinical systems within the NHS in England MUST conform to the NHS Number input and display standard by 01 December 2015. Patient safety risks associated with the NHS Number input and display standard are specified in the safety case accompanying the standard. Organisations that develop, supply, deploy and use clinical systems must identify and manage any risks in current systems and mitigate implementation risks for which responsibility for mitigation has been transferred to the supplying or deploying organisation. The process for managing safety risks is detailed in NHS information standards for the application of patient safety risk management to the manufacture, deployment and use of health software. The standard can also be used to inform design of user interfaces in other systems and devices provided any safety risks are identified and managed, for example, risks related to reduced screen size.		
Other Datasets / Returns Affected: None		
Related DSCNs: ▪ DSCN 18/2009 - Application of Patient Safety Risk Management to the Deployment and Use of Health Software. ▪ DSCN 14/2009 - Application of Patient Safety Risk Management to the Manufacture of Health Software. ▪ AN/0904 – Demographics Display and Input for Clinical Systems in the NHS in England. ▪ DSCN 09/2010 – Patient Banner for Clinical Systems within the NHS in England. ▪ DSCN 31/2008 – NHS Number Standard for General Practice (England) v1.1. ▪ DSCN 32/2008 – NHS Number Standard for Secondary Care (England).		
Impact of Change:		
Service:	Major	System Suppliers: Major
The Information Standards Board for Health and Social Care (ISB) is responsible for approving information standards.		

DATA SET CHANGE NOTICE

Reference No:	DSCN 03/2010
Version No:	2.0
Subject:	NHS Number Input and Display for Clinical Systems within the NHS in England
Type of Change:	Introduction of a new approved Information Standard
Implementation Date:	01 December 2015
Business Justification:	To make display, input and user interaction consistent across all clinical systems within the NHS in England in order to increase patient safety by maximising clinical utility and minimising reading and transcribing errors.

Introduction

Displaying and inputting unambiguous information in a consistent format is a core element in ensuring effective patient care. It is vital that healthcare IT users correctly interpret and input information relating to all aspects of patient care and management.

This user interface standard specifies the rules for the display and input of NHS Numbers for clinical systems.

The standard has been developed as part of the Common User Interface (CUI) programme within the Technology Office of the Department of Health Informatics Directorate (DHID). This programme is addressing the safety risks posed by variation across the NHS in the way information is input and displayed. Its aim is to support healthcare IT users in correctly inputting and interpreting information relating to all aspects of patient care and management.

Background

Accurate identification of patients is vital to ensure that the correct patient receives the right care. This is a very frequent task and must necessarily be repeated many times in one day, for example, every time a patient presents themselves to a receptionist or is to have a drug administered. In addition to being accurate, the patient-identification task must therefore also be performed efficiently.

The NHS Number is the national unique patient identifier and is fundamental to the UK National Programme for IT (NPfIT). It is a national unique identifier that makes it possible to share electronic patient information across the whole of the NHS safely, efficiently and accurately. As such, it is the key to unlocking services such as the NHS Care Records Service, Choose and Book or the Electronic Prescription Service. The NHS Number also provides the means to efficiently integrate a patient's data from disparate registers in a trust, combining these data into a master register. Additional checks with the full set of patient information, such as first name, last name, gender, date of birth and address, are used for greater accuracy when matching for clinical purposes.

Encouraging widespread use of the NHS Number is an important way of reducing the number of occasions when patients are incorrectly identified, which is a cause of clinical risk recognised by the National Patient Safety Agency (NPSA), [please refer to the NPSA Safer Practice Notice for NHS Number](#). In addition to wristbands, the DHID recommends, as a minimum, that the NHS Number is used on the following items:

- Clinical records – detail and summaries.
- Referrals to other organisations – including electronic bookings.

- Clinic appointment letters.
- Test requests.
- Test results.
- Samples.
- Discharge notices.

The NHS Number is a ten-digit number assigned to every one of the 56 million patients registered with GPs in England and Wales. The first nine digits are the identifier, with the tenth used as a check digit to confirm the number's validity. It will be read from screen displays and be correlated with information in different media, for example, wristbands, paper records, and forms, to check that these show the same identifier. Unambiguous display enhances patient safety and application usability by enabling rapid correlation with paper records and forms on which the identifier is printed. Therefore, it should be presented in an easy-to-read, correct and consistent format across all media.

Details of the Standard

The standard is published with associated guidance in a Common User Interface Design Guide. It is applicable to user interfaces displayed on desktop or laptop computers. It is assumed that, at a minimum, these computers are capable of operating at a minimum display resolution of 1024 x 768, and have a keyboard and pointing device.

The following items are covered in the guidance:

- Input of a NHS Number.
- Display of an NHS Number in the required 3 3 4 format (e.g. 943 476 2812).

Aspects that are **out of scope**:

- Storage format
- Validation
- Verification
- Multi-language
- Display
- Other patient identification
- Bar code
- Reduced-size form
- Data storage and transmission
- Data history and provenance
- Form design
- Pseudonymisation

Safety Case

Clinical safety requirements and objectives related to the input and display of NHS Numbers in clinical systems are documented in the safety case that accompanies this standard (www.isb.nhs.uk/documents/cui). Organisations that develop, supply, deploy and use clinical systems are required to identify and manage any risks in current systems until the standard can be implemented. They must also mitigate those live implementation risks outlined in the safety case for which responsibility for mitigation has been transferred to the supplying or deploying organisation.

The process for assessing and managing safety risks is detailed in NHS information standards for the application of patient safety risk management to the manufacture, deployment and use of health software (<http://www.connectingforhealth.nhs.uk/dscn/dscn2009/data-set-change/>), i.e.:

- Application of Patient Safety Risk Management to the Manufacture of Health Software (DSCN 14/2009); and
- Application of Patient Safety Risk Management to the Deployment and Use of Health Software (DSCN 18/2009).

Conformance

For full conformance to be demonstrated, the elements marked as 'Mandatory' in the Common User Interface Design Guide must be complied with and there must have been mitigation of those live implementation risks outlined in the safety case for which responsibility for mitigation has been transferred to the supplying or deploying organisation.

Timescales for Implementation

FRAMEWORK	Health and Social Care Personnel	Organisation	IT Suppliers
Effective Date "may use"	Immediate	Immediate	Immediate
Implementation Date "must use"	When available in system	At next major system release	At next major system release
Conformance Date "must be used effectively and assessed for use"	01 December 2015	01 December 2015	01 December 2015
Superseded Date (of prior standard) "stop using prior standard"	Not applicable	Not applicable	Not applicable

Sponsor Details

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Technology Office

Further Information and Support

- CUI NHS Number Input and Display Design Guide – www.cui.nhs.uk
- Clinical Safety Case - www.isb.nhs.uk/documents/cui
- CUI website - www.cui.nhs.uk
- Standards documentation – www.isb.nhs.uk/documents/cui
- Queries - cui.stakeholder@mailbox.nhs.net

Appendix

This DSCN must be read in conjunction with the Clinical Safety Case and Closure Report for Demographics / Information Input and Display, which can be found at the following link: www.isb.nhs.uk/documents/cui

Table Notes:

1. Relevant organisations are those organisations as defined in the standard who must take direct action to implement the standard
2. IT Suppliers are all suppliers to the organisations listed at ¹ who supply functionality pertinent to that standard
3. **Effective Date** is the date from which a new standard can be used but may not be mandatory. This might facilitate piloting, for example, or enable time for system functionality development. At this point, **you "may use" the standard.**
4. **Implementation Date** is the point from which the new standard becomes mandatory. Ideally, it inherently implies organisations use appropriate systems i.e. the date is the same for organisations and suppliers. However, there may be circumstances where interim workarounds are required i.e. the date is different for organisations and suppliers. At this date, **you "must use" the standard.** Where the standard demands data is submitted centrally, sub components of implementation date (and possibly 'effective date') are:
 5. **Collection Start Date** – this is the date collection of data must begin
 6. **First Submission Date** – this is the date of first submission of data centrally

7. **Reporting Period / Submission Cycle** – If the standard calls for further collection and submission at defined intervals, this cell provides text of the reporting period (e.g. calendar month, financial year) and the submission cycle (e.g. submit data monthly on the 10th working day of the subsequent month).
8. **Conformance Date** is the date from which the service and IT system suppliers must use the standard as envisaged i.e. using appropriate IT solutions rather than interim workarounds and, if the standard requires it, an independent, authoritative body or legitimate internal audit would conduct a conformity assessment with the expectation of full conformance by all relevant parties. It is the ***“must use standard effectively and assessed for use”*** date
9. **Superseded Date** of the prior standard sets the date at which the prior standard is replaced by the new standard i.e. the prior standard must no longer be used. This date will apply only where there was a pre-existing standard made redundant by the new standard. It might be different from preceding dates in the framework if, for example, a new and old standard run in parallel for a period. It is the date from which you ***“stop using the prior standard”***.